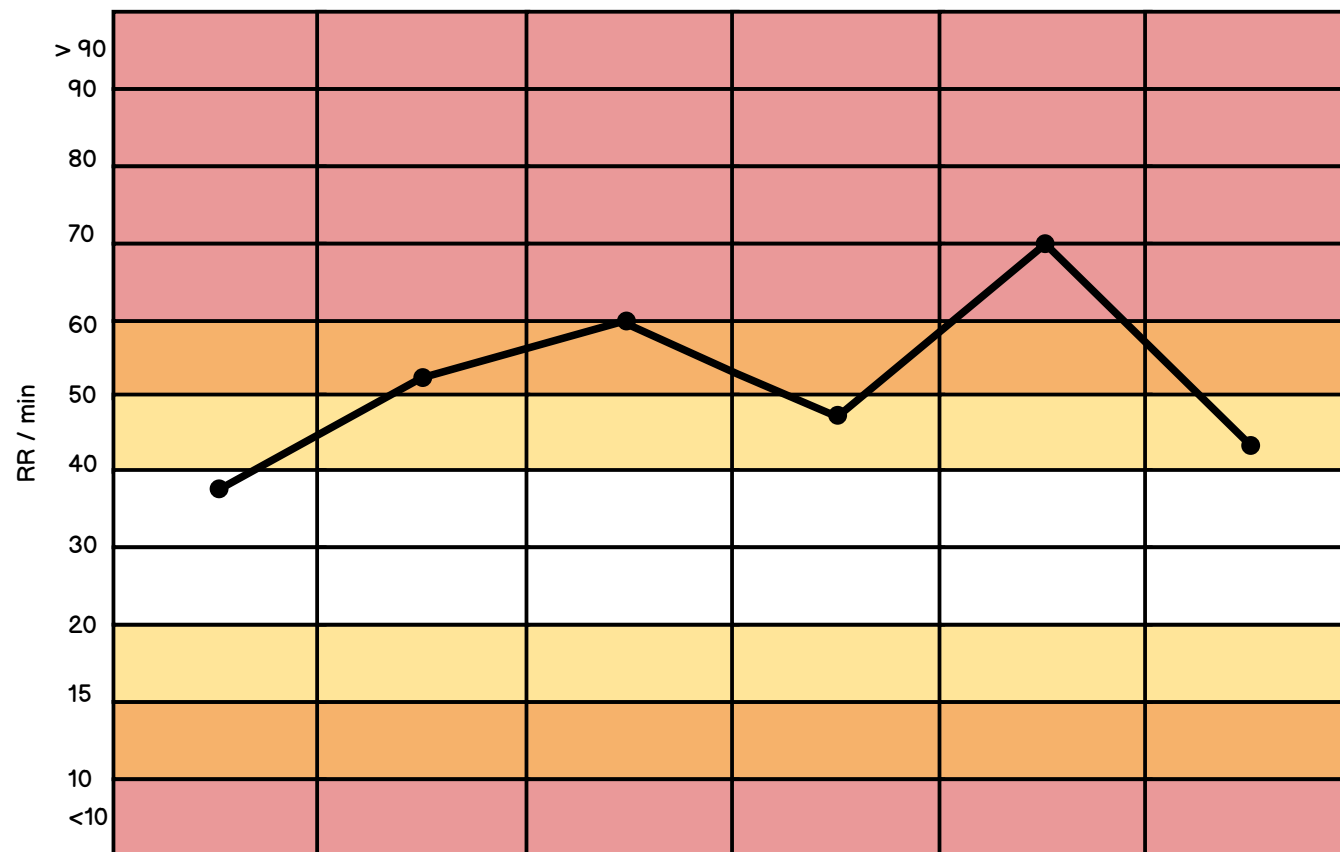


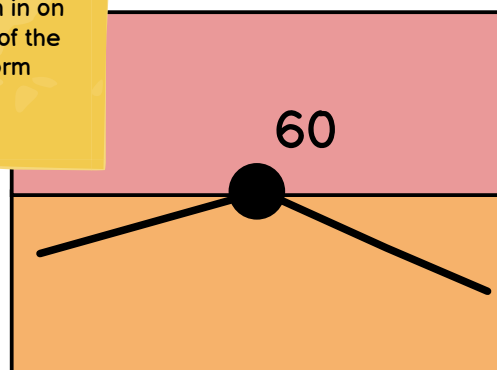
PEWS Wireframes v2.0

W1.1 Respiratory Rate

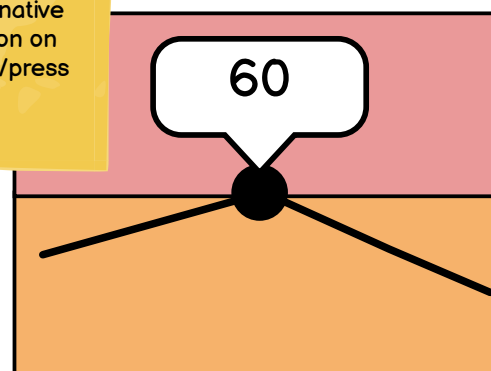


This wireframe uses the age 5 - 12 scores, different ages would score differently

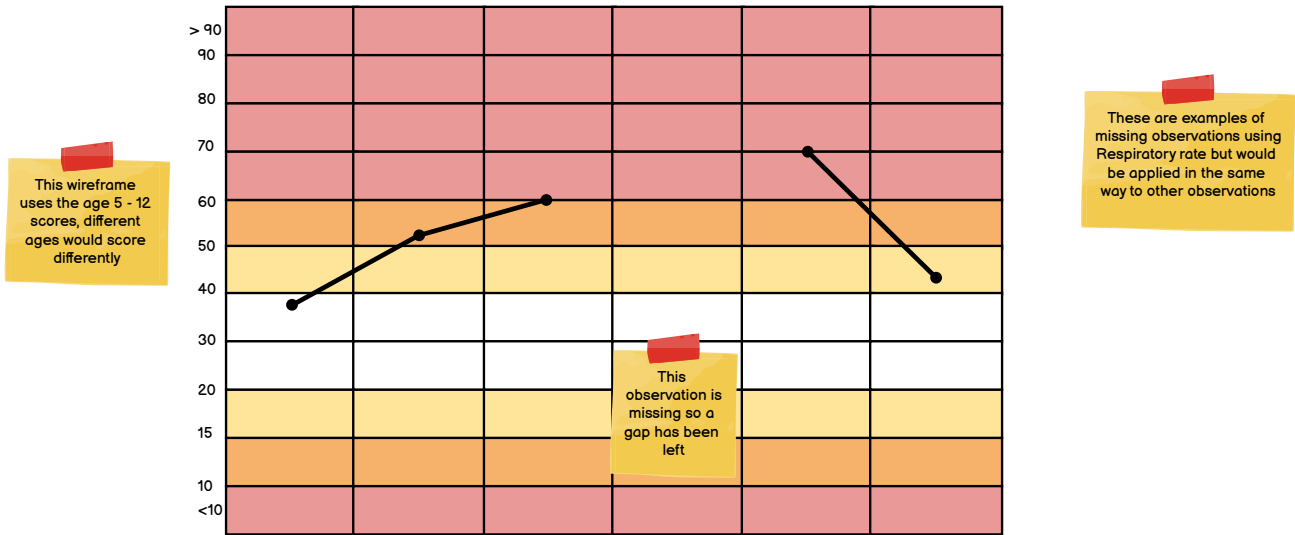
Zoom in on part of the form



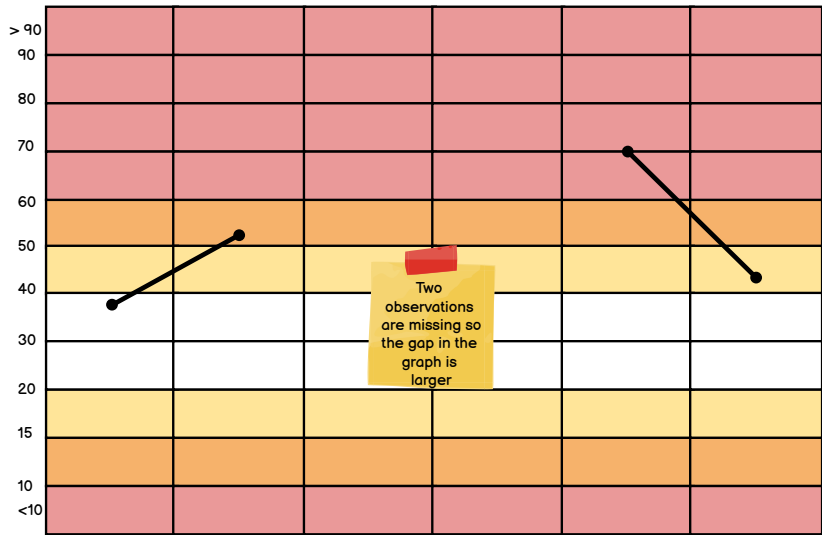
Alternative option on click/press



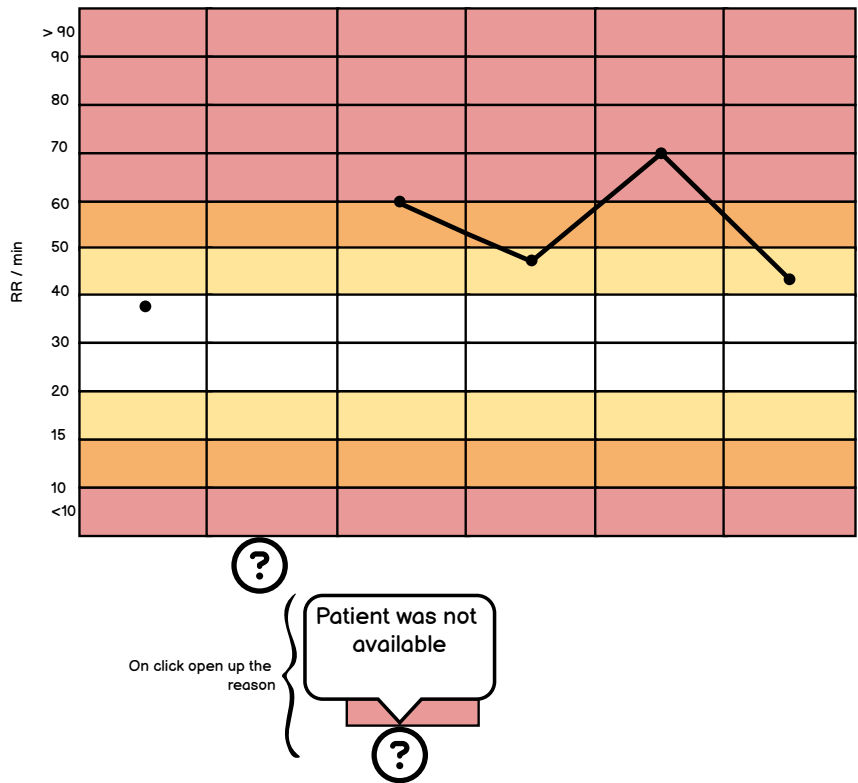
W2.1 Respiratory Rate One missing observation



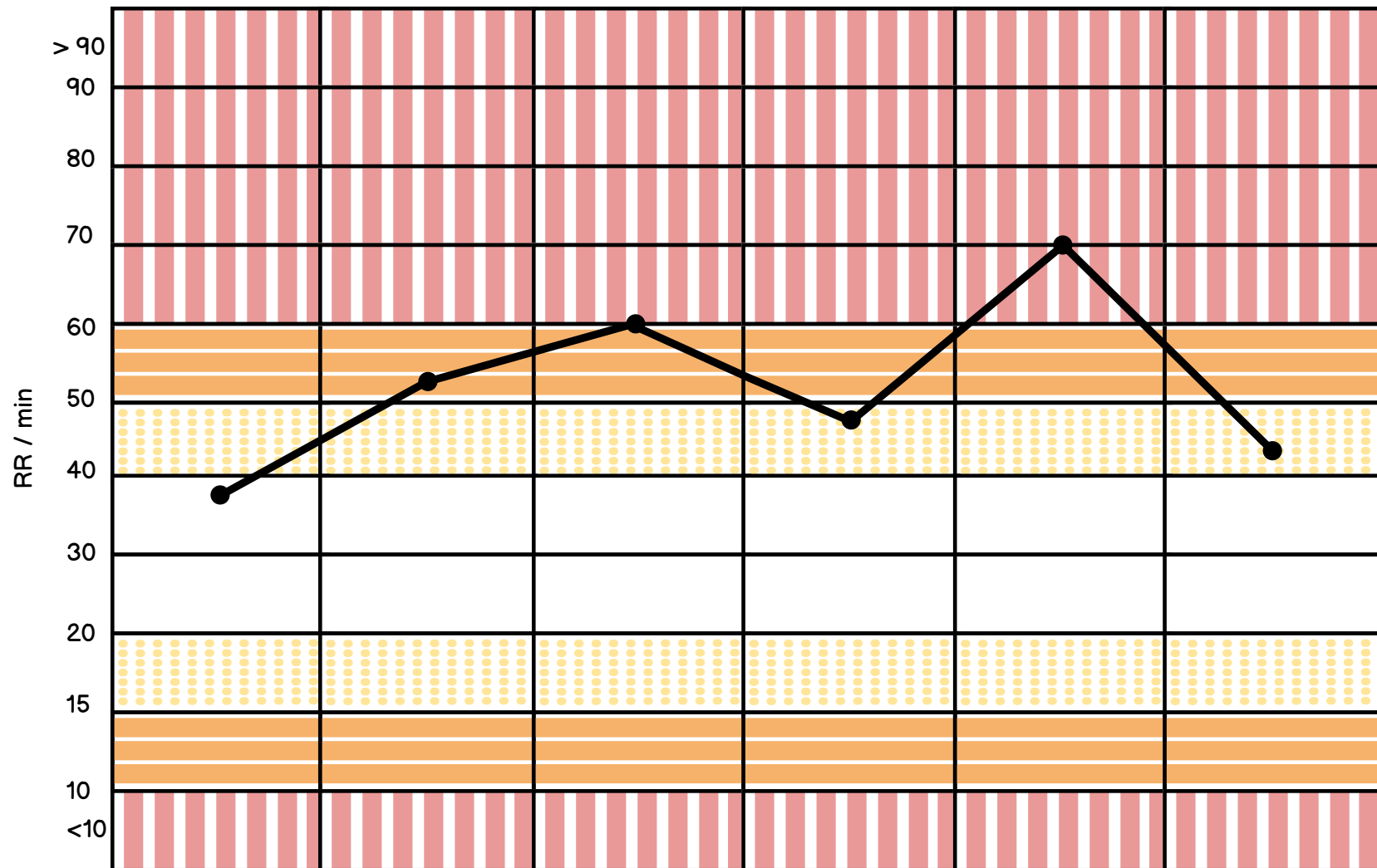
W2.2 Respiratory Rate Two missing observations



W2.4 Respiratory Rate Example of reason for missing observation



W3.1 Respiratory Rate Colour Blind version



This is an example of a colour blind version of respiratory rate. Other patterns could be used. This would be applied in a similar way to other observations

W4.1 Respiratory Distress Option 1:

Distress Observed	None	Severe	Severe	Moderate	Moderate	Mild
-------------------	------	--------	--------	----------	----------	------

W4.2 Respiratory Distress Option 2:

Distress Observed	Severe		●	●				
	Moderate				●	●		
	Mild							●
	None	●						

W5.1 Oxygen Saturation Option 1:

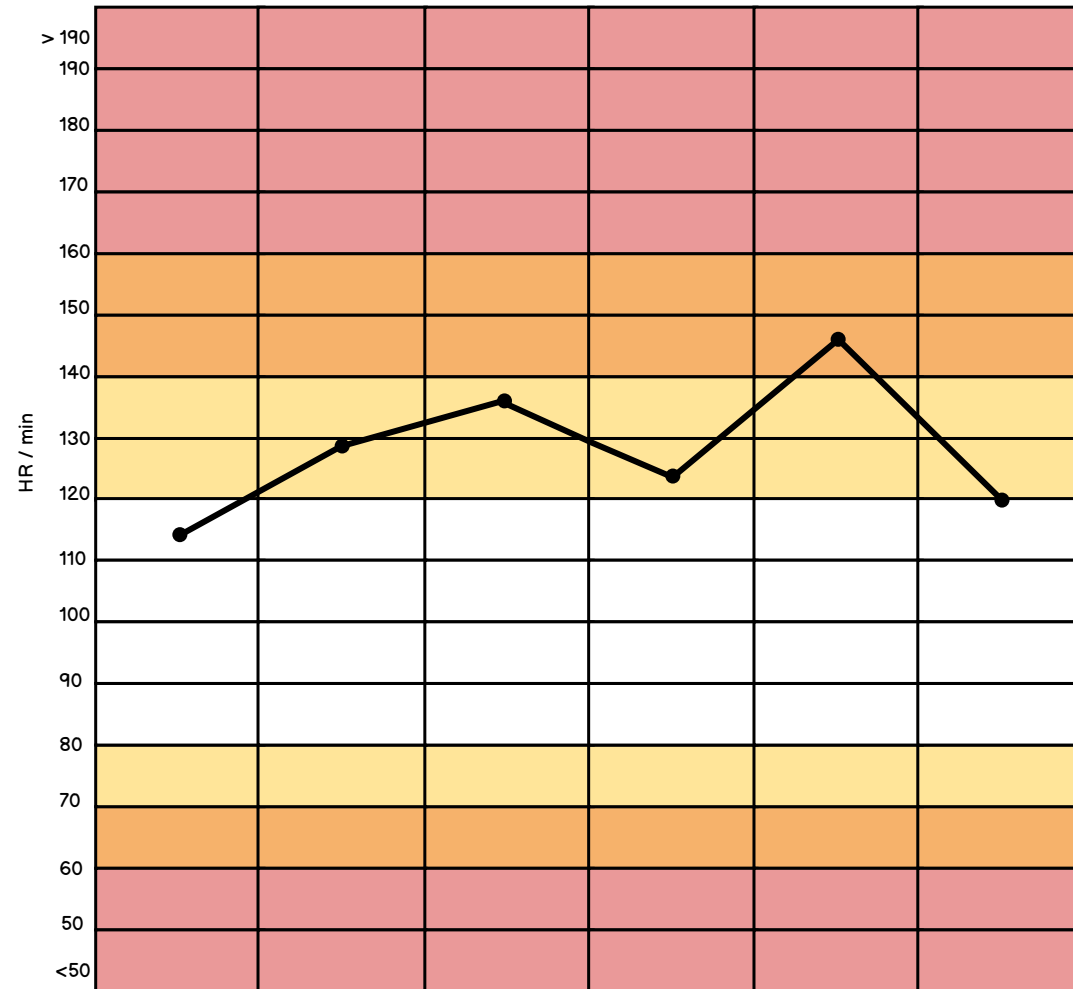
SpO2	97%	89%	90%	92%	93%	94%
------	-----	-----	-----	-----	-----	-----

W5.2 Oxygen Saturation Option 2:

SpO2	$\geq 95\%$					
	92% - 94%					
	$\leq 91\%$					

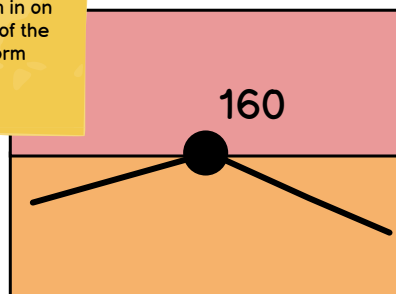
This version would still have an option to get the numbers (e.g. on click/press)

W6.1 Heart Rate

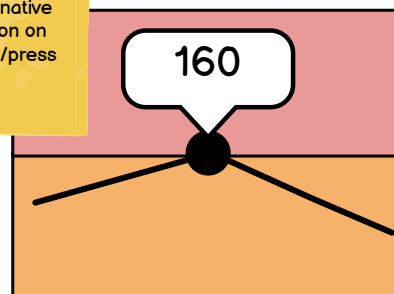


This wireframe uses the age 5 - 12 scores, different ages would score differently

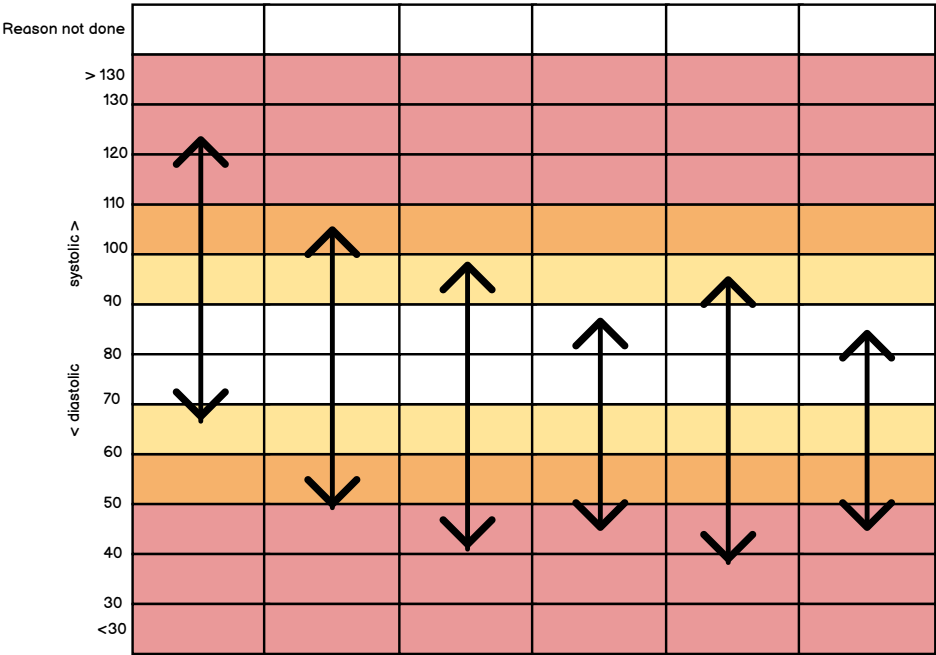
Zoom in on part of the form



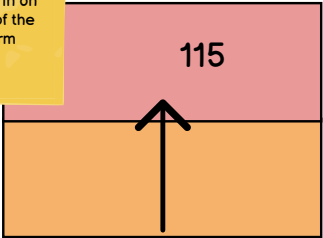
Alternative option on click/press



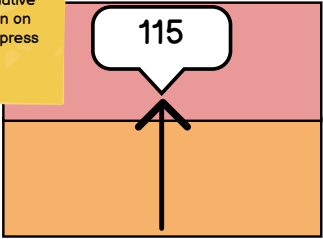
W7.1 Blood Pressure Option 1



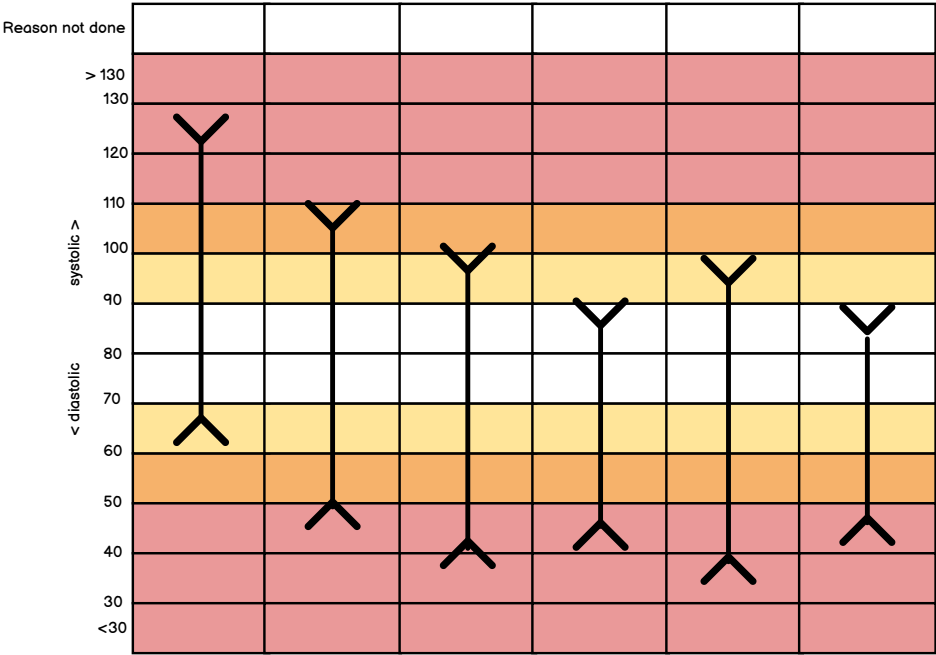
Zoom in on part of the form



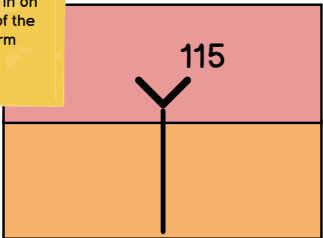
Alternative option on click/press



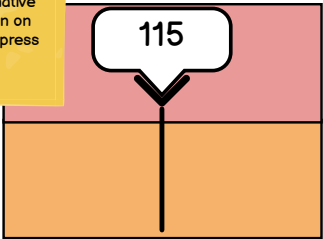
W7.2 Blood Pressure Option 2



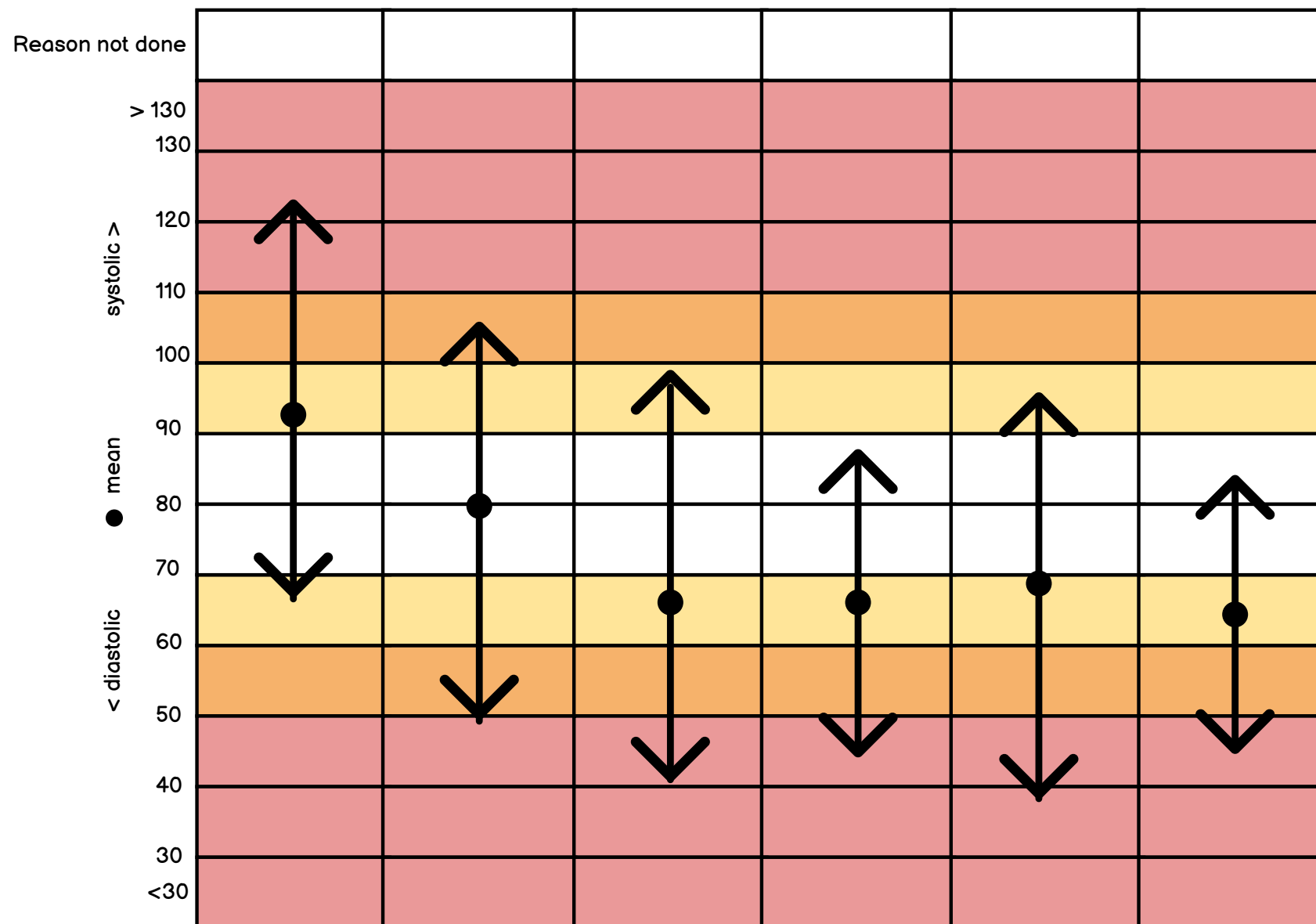
Zoom in on part of the form



Alternative option on click/press



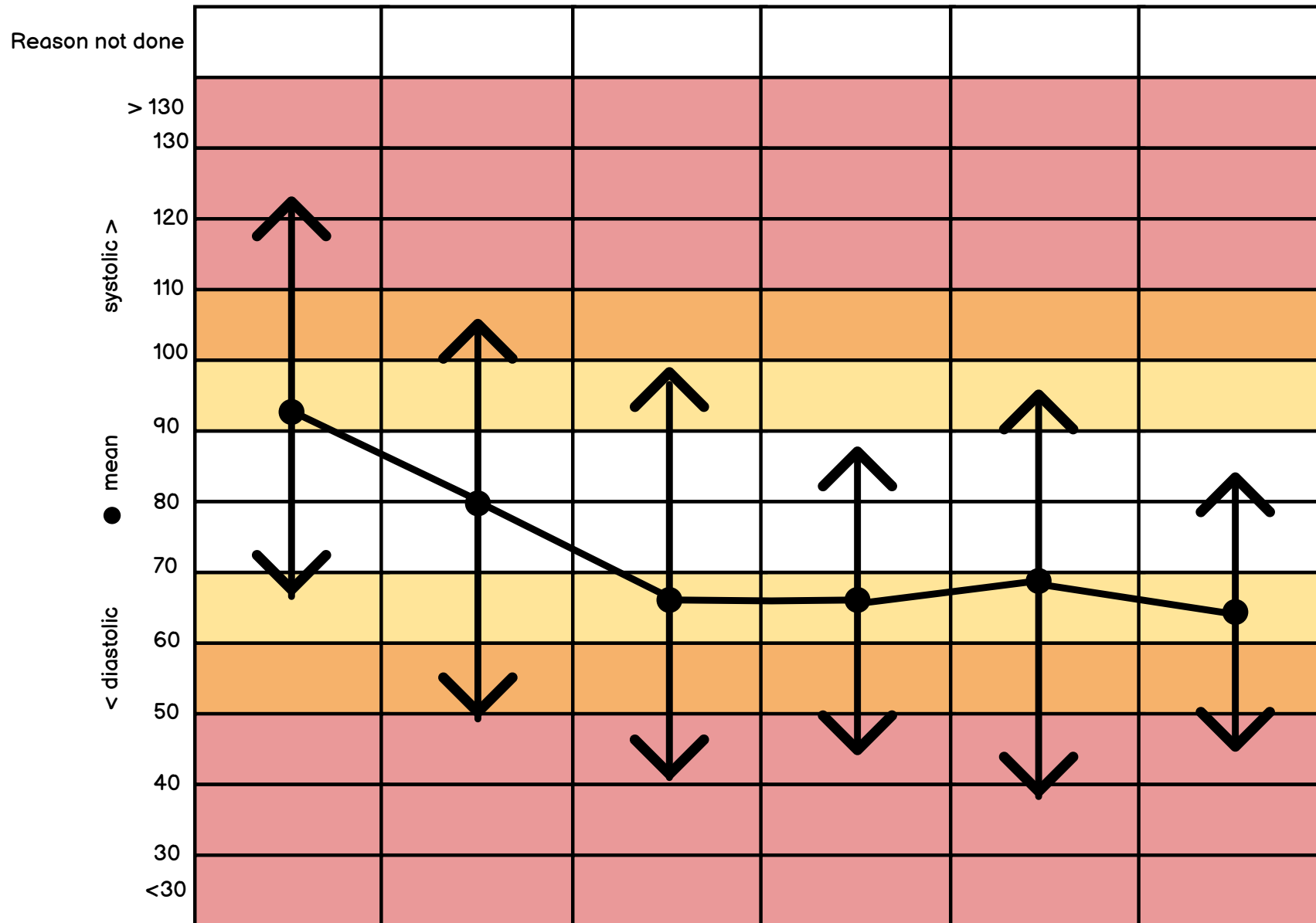
W8.1 Blood Pressure with means



This could be done with either arrow configuration, one is used to avoid repetition

This wireframe uses the age 5 - 12 scores, different ages would score differently

W9.1 Blood Pressure with means graphed



This could be done with either arrow configuration, one is used to avoid repetition

This wireframe uses the age 5 - 12 scores, different ages would score differently

W10.1 CRT Option 1:

CRT in seconds

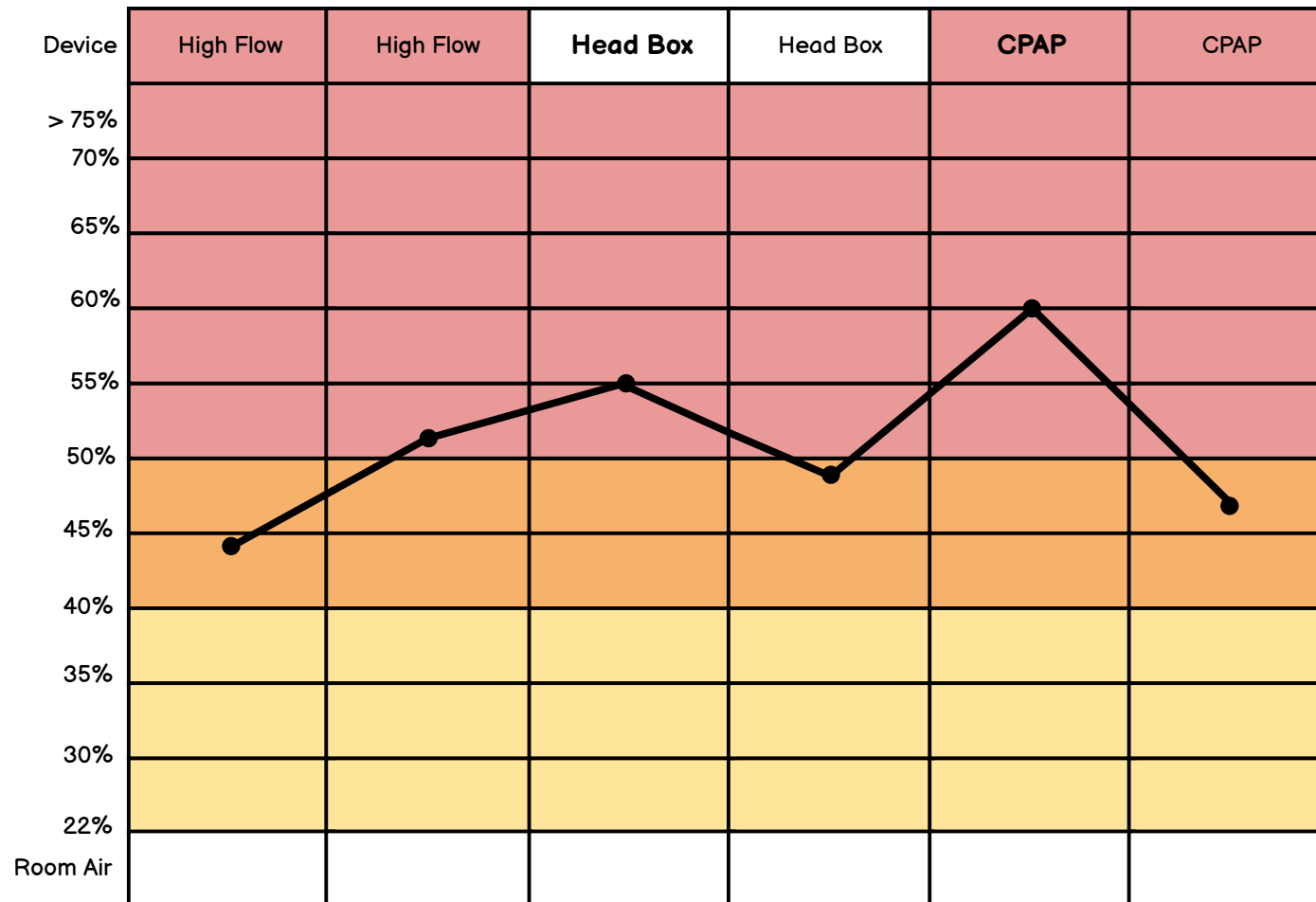
2	1	2	3	4	3
---	---	---	---	---	---

W10.2 CRT Option 2:

CRT
≥ 3 secs
≤ 2 sec

			3	4	3
2	1	2			

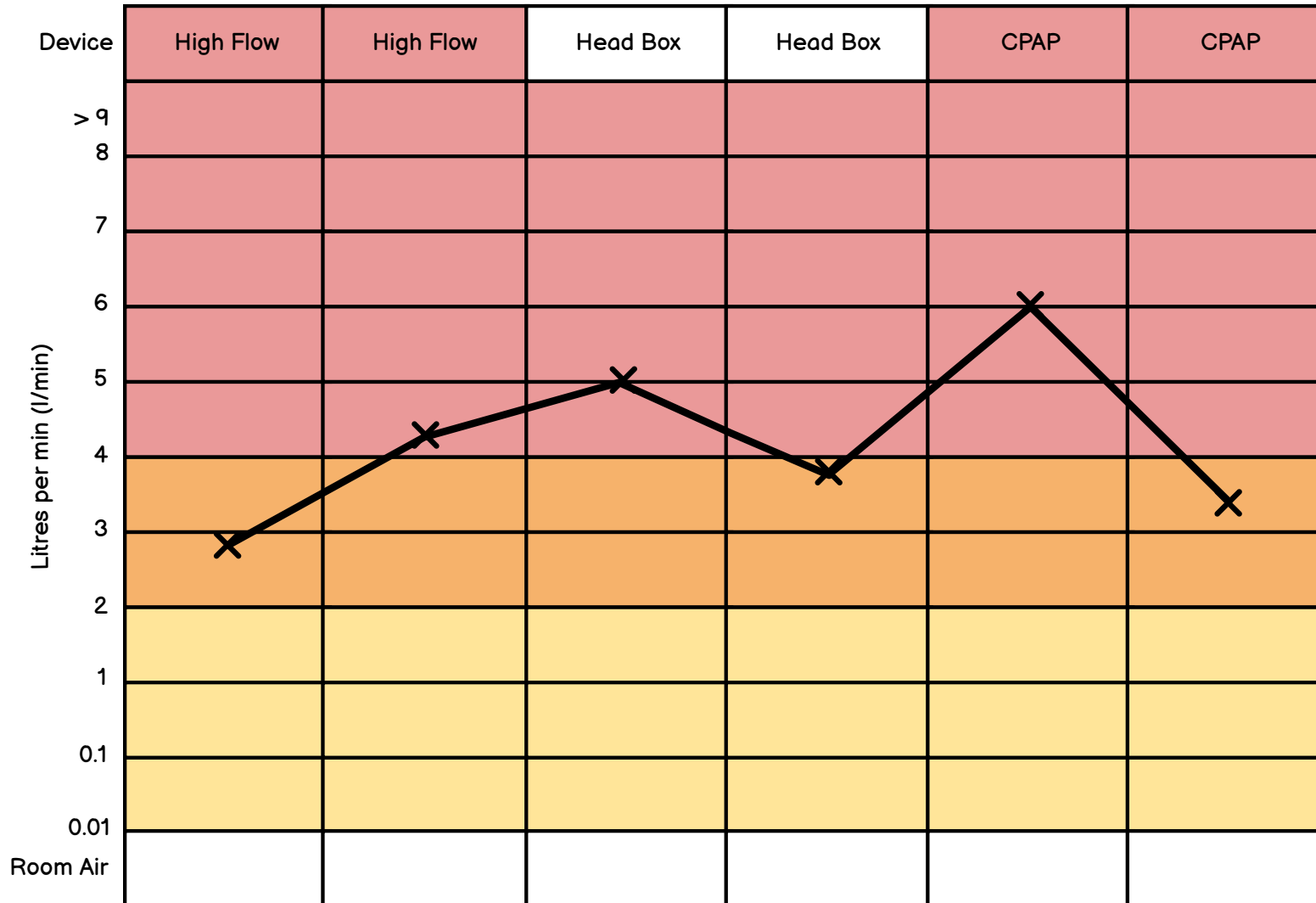
W11.1 Respiratory Support using percentages



In this example the device is bolded when it changes to make clear the device has changed to meet requirement C8.6. Other formats would be acceptable as long as it was clear the device had changed.

This wireframe uses the age 5 - 12 scores, different ages would score differently

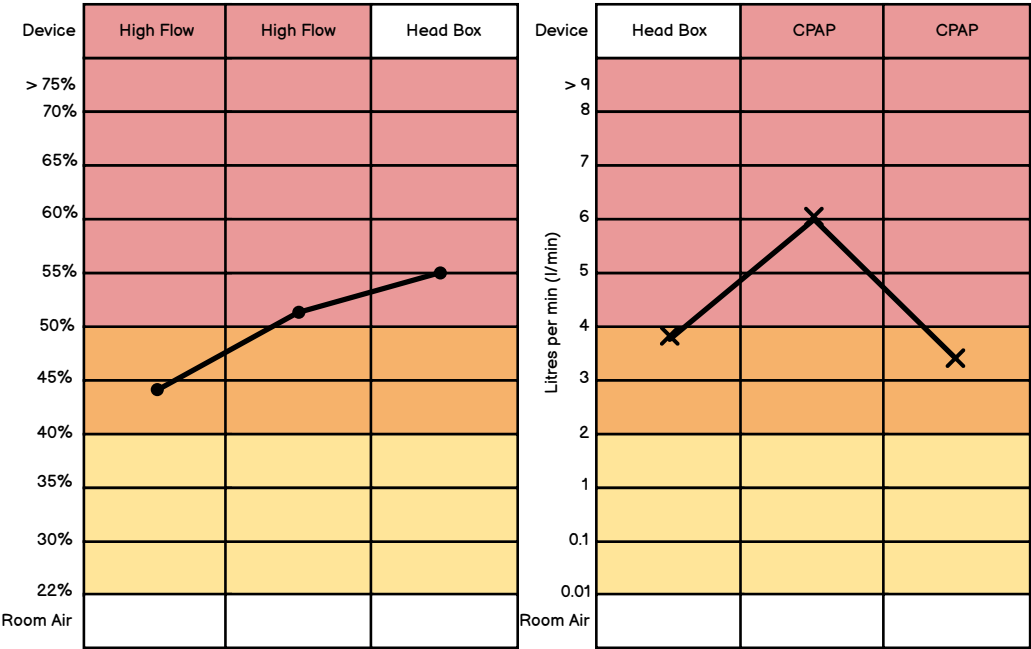
W12.1 Respiratory Support using litres per min



This wireframe uses the age 5 - 12 scores, different ages would score differently

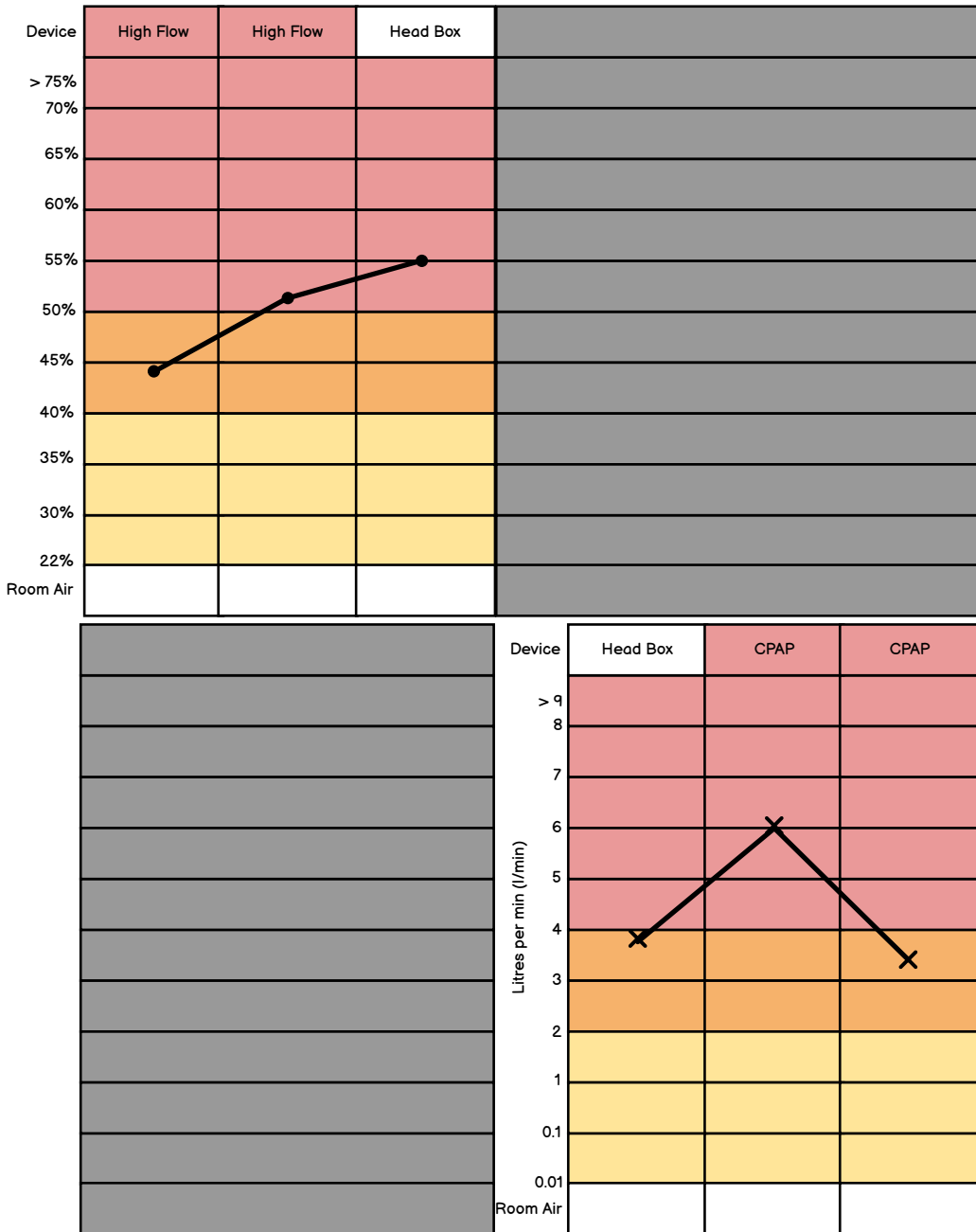
Respiratory Support switching methods

W13.1 Option one - Continuous chart with new Y axis



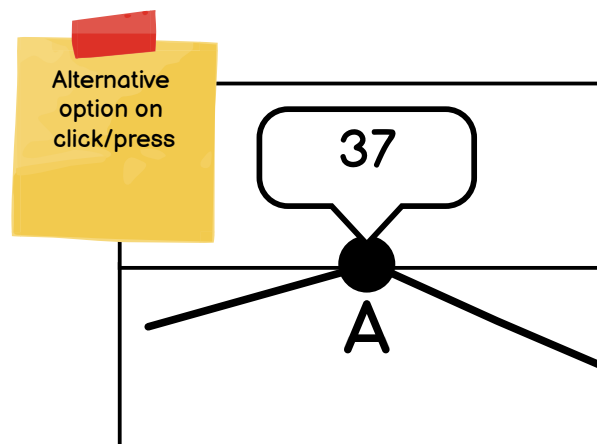
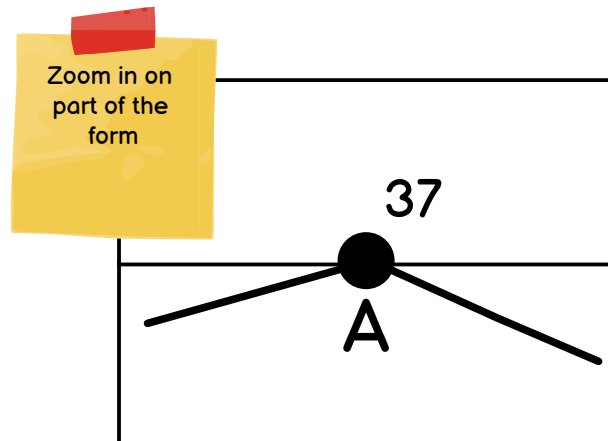
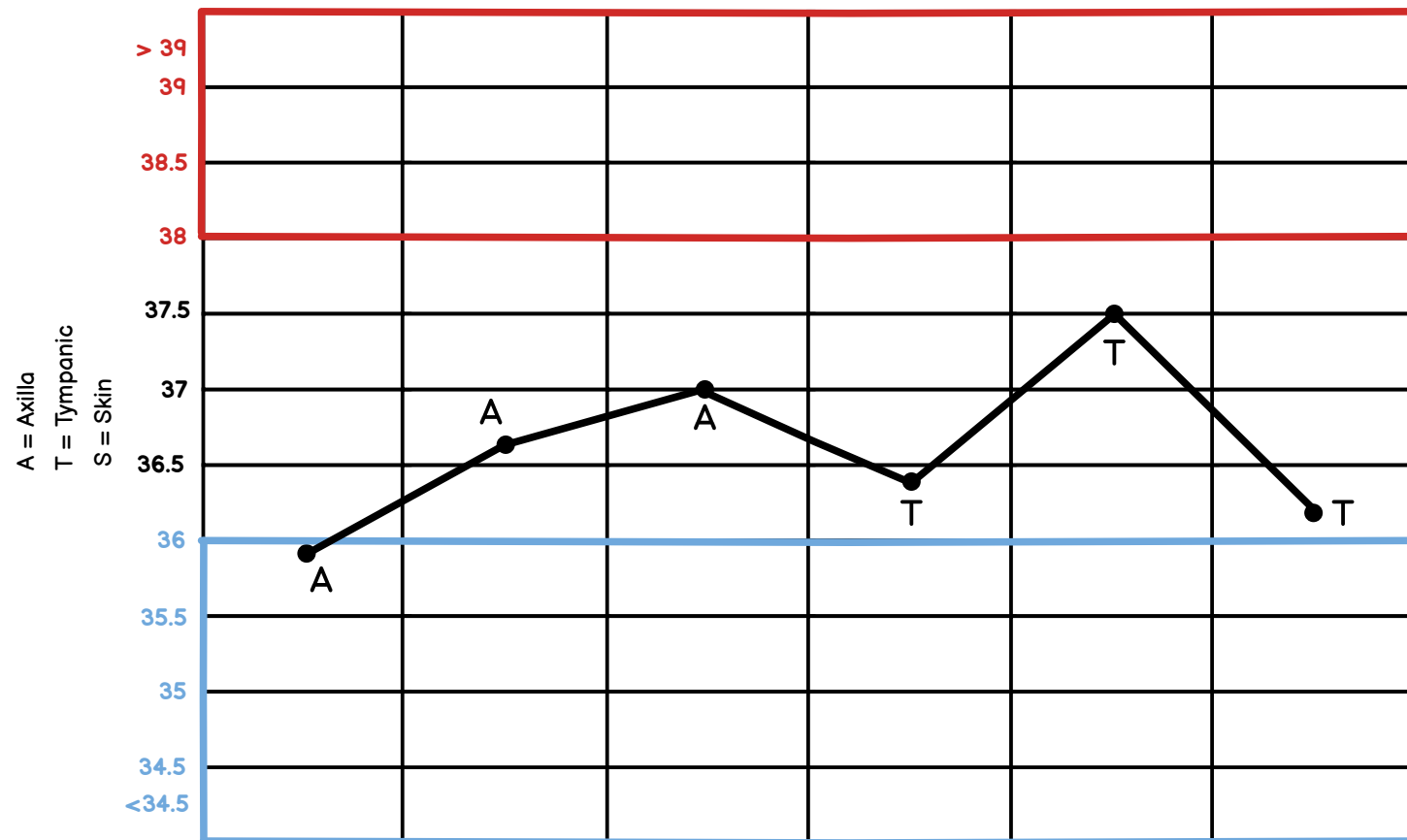
This wireframe uses the age 5 - 12 scores, different ages would score differently

W13.2 Option two - Displaced new chart



This wireframe uses the age 5 - 12 scores, different ages would score differently

W14.1 Temperature



Alerts

W15.1 Patient escalation alert


PEWS Escalation Level - High


The patient's PEWS Score has been rated as High.
Below is the guidance on a high escalation

Immediate review by Nurse in Charge for potential escalation
Call for 'Rapid Review': Medical incl. airway skills ST3+ or equivalent and
outreach nurse (if available or equivalent)
Stabilisation plan to be discussed with consultant
Senior nurse to feedback plan to parents
Medical review within 15 mins
Minimal Observations - every 30 minutes and continuous monitoring of Respiratory Rate / Oxygen Saturation / ECG
GCS Recording if change in AVPU

Please select whether to use the plan in place, or to escalate to High now

This is an example of the alert, though as long as it's clear to the clinician it will work

W15.2 Patient level increasing alert


PEWS Escalation Level - High




This patient has increased from MEDIUM to HIGH. This increase in level should be escalated so that they can be reviewed.



The patient's PEWS Score has been rated as High.
Below is the guidance on a high escalation

Immediate review by Nurse in Charge for potential escalation
Call for 'Rapid Review': Medical incl. airway skills ST3+ or equivalent and
outreach nurse (if available or equivalent)
Stabilisation plan to be discussed with consultant
Senior nurse to feedback plan to parents
Medical review within 15 mins
Minimal Observations - every 30 minutes and continuous monitoring of Respiratory Rate / Oxygen Saturation / ECG
GCS Recording if change in AVPU

Please select whether to use the plan in place, or to escalate to High now

Break out Escalation options

These are some examples of how a clinician could break out of observations to do an escalation. This would satisfy requirement U17.2

W16.1 Option 1 - Escalation Menu

Save form	New form	Exit form	View different patient	ESCALATE CURRENT PATIENT NOW
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In this option the normal top of the page menu for the software has an extra button to escalate the patient outside of the normal routine. The names of the other menu options are just examples. The important part is the escalation button being visually distinct

W16.2 Option 2 - Escalation Floating button



This floating button would display in the corner of the display while observations are being conducted. Clicking it would let you break out and escalate. The button would float so that it will always show in the corner even if you scroll

W16.3 Option 3 - Escalation banner



The banner would show across the page as you scroll up and down. This would let you break out at any point to escalate.

W17.1 Respiratory Distress Selection

Select observed respiratory distress:

☐ No distress observed

Mild Respiratory distress

☒ Nasal flaring

☐ Subcostal recession

Moderate Respiratory distress

☐ Head bobbing

☐ Tracheal tug

☒ Intercostal recession

☐ Inspiratory or expiratory noises

Severe Respiratory distress

☒ Sternal recession

☐ Grunting

☐ Exhaustion

☐ Impending respiratory arrest

Skip observation

Submit

This wireframe uses the age 1 - 4 categories, different ages would score differently

The clinician could tick or untick various options and submit. The system will then score the worst/highest of the options.

W18.1 Parent / Carer response Selection

Select parent / Carer response:

- ☒ Worse
- ☐ Same
- ☐ Better
- ☐ Parent / Carer Asleep
- ☐ Parent / Carer Unavailable

Skip observation

Submit

The clinician could tick or untick one option then submit. If they select Worse they'll get a secondary escalation screen

W18.2 Parent / Carer escalation

Select parent / Carer escalation levels:

The parent / carer has responded that their child is doing worse. Please select which escalation level you'd like to select based on their view of the child

Do Not Escalate

Escalate to Low Now

Escalate to Medium Now

Escalate to High Now

Escalate to Emergency Now

W19.1 Clinical intuition response Selection

Clinical Intuition response:

☒ Yes - Escalate

☐ No

☐ I'm not sure

Skip observation

Submit

The clinician could tick or untick one option then submit. If they select Yes they'll get a secondary escalation screen

W19.2 Clinical Intuition escalation

Select clinical escalation levels:

You have responded that you are concerned about the child.
Please select which escalation level you'd like to select based on your view of the child

Do Not Escalate

Escalate to Low Now

Escalate to Medium Now

Escalate to High Now

Escalate to Emergency Now

W20.1 Respiratory Support Selection

Select level of respiratory support:

- ☐ No respiratory support
- ☐ Nasal prongs
- ☐ Face mask
- ☐ Head box
- ☐ Non-rebreather
- ☒ High Flow
- ☐ BiPAP
- ☐ CPAP

Skip observation

Submit

The clinician could tick or untick one option then submit. The system will then know how to score.

W21.1 Sepsis suspicion question

New suspicion of sepsis or septic shock?

- ☒ Yes - New suspicion of sepsis
- ☐ Yes - New suspicion of septic shock
- ☐ No new suspicion

Skip observation

Submit

Selecting sepsis will escalate to medium, selecting septic shock will escalate to high

W21.2 AVPU question

Patient AVPU response:

- ☐ A - Alert
- ☐ V - Responsive to Voice
- ☒ P - Responsive only to Pain
- ☐ U - Unresponsive
- ☐ Asleep

If the patient scores V, P or U you should do a Glasgow Coma Score sheet

Skip observation

Submit

Selecting V will escalate to High, selecting P or U will escalate to Emergency. Selecting A or asleep will not escalate.

A system with digital GCS could auto trigger the form

W22.1 Escalation Trigger Criteria Selection

Select level of respiratory support:

- ☐ Specific Concern (SC)
- ☐ Carer Question (CQ)
- ☐ Clinical Intuition (CI)
- ☐ PEWS Score (P)

No Escalation

Submit

Generally this would be set automatically if only one areas triggers.

If this isn't possible then a selection box should be used.

Even with automation if two or more areas trigger you may wish to present a selector so the clinician can pick which one to select.